**SID No** : **47020143**
**Branch** : **MUMBAI**
**Mrs. BHARATI RAJE**
Age / Sex : 32 Y / Female
Ref. By : **METRO PATHOLOGY LAB-UPPER THANE**
Doctor : Dr.BHAKTI MHATRE

**Patient ID** : **4700263328**

Collected Date : 27/07/2025 / 17:13
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**Final Test Report**



| Specimen | Test Name | Result | Units | Reference Range / Method |
|---|---|---|---|---|
| **HAEMATOLOGY** | | | | |
| **Health Package - 4** | | | | |
| **Complete Hemogram** | | | | |
| EDTA BLOOD | Haemoglobin | 13.0 | g/dL | 12.0 - 15.0 (Colorimetric) |
| EDTA BLOOD | Total WBC count | 5960 | cells/cumm | 4000 - 10000 (Electrical Impedance) |
| **Differential Count** | | | | |
| EDTA BLOOD | NEUTROPHILS | 61.00 | % | 40 - 70 (DHSS) |
| EDTA BLOOD | LYMPHOCYTES | 28.70 | % | 20 - 45 (DHSS) |
| EDTA BLOOD | EOSINOPHILS | 1.80 | % | 1.00 - 7.00 (DHSS) |
| EDTA BLOOD | MONOCYTES | 8.20 | % | 2.00 - 7.00 (DHSS) |
| EDTA BLOOD | BASOPHILS | 0.30 | % | 0 - 1 (DHSS) |
| EDTA BLOOD | Red Blood Cell (RBC) Count | 4.63 | million/cumm | 3.80 - 4.80 (Electrical Impedance) |
| EDTA BLOOD | PCV | 40.0 | % | 37.0 - 47.0 (Calculated) |
| EDTA BLOOD | MCV | 86.4 | fl | 83.0 - 101.0 (Electrical Impedance) |
| EDTA BLOOD | MCH | 28.1 | % | 27.0 - 32.0 (Calculated) |
| EDTA BLOOD | MCHC | 32.5 | % | 31.5 - 34.5 (Calculated) |
| EDTA BLOOD | Platelet count | 339000 | cells/cumm | 150000 - 410000 (Electrical Impedance) |
| EDTA BLOOD | Absolute Neutrophils | 3630 | cells/cumm | 2000 - 7000 (Calculated) |

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|---|---|---|---|---|
| EDTA BLOOD | Absolute Lymphocytes | 1710 | cells/cumm | 1000 - 3000 (Calculated) |
| EDTA BLOOD | Absolute Eosinophils | 110 | cells/cumm | 20 - 500 (Calculated) |
| EDTA BLOOD | Absolute Monocytes | 490 | cells/cumm | 200 - 1000 (Calculated) |
| EDTA BLOOD | Absolute Basophils | 20 | cells/cumm | 20 - 100 (Calculated) |
| EDTA BLOOD | RDW - CV | 13.5 | % | 11.0 - 16.0 (Calculated) |
| EDTA BLOOD | RDW - SD | 37.0 | fl | 37.0 - 54.0 (Automated) |
| EDTA BLOOD | MPV | 7.6 | fl | 9.0 - 13.0 (Electrical Impedance) |
| EDTA BLOOD | PCT | 0.26 | % | 0.17 - 0.38 (Automated) |
| EDTA BLOOD | PDW | 10.4 | fl | 9.0 - 17.0 (Automated) |

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| **BIOCHEMISTRY** | | | | |
| **Health Package - 4** | | | | |
| **HbA1c** | | | | |
| EDTA BLOOD | Glycosylated Haemoglobin (HbA1c) | 5.4 | % | Adult<br>Normal : < 5.7<br>Prediabetic : 5.7-6.4<br>Diabetic : >= 6.5<br><br>A1C Goals<br>Reasonable Goal : <7<br>More stringent goal : <6.5<br>Less stringent goal : <8.0<br>(ADA, 2019)<br>(immunoturbidimetric)<br>(Immunoturbidimetric) |
| EDTA BLOOD | Estimated Average Glucose (eAG) | 107 | mg/dL | |
| **Lipids Profile** | | | | |
| Serum | Cholesterol, Total | 163.4 | mg/dL | Adult (NCEP ATP-III)<br>Desirable : <200<br>Borderline high : 200 - 239<br>High : >239 (Enzymatic : CHOD-PAP) |

Note: Above Biological interval is based on 9 to 12 hours fasting.

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| Specimen | Test Name | Result | Units | Reference Range / Method |
|---|---|---|---|---|
| Serum | Triglycerides | 63.60 | mg/dL | Adult (NCEP ATP-III)<br>Normal : <150<br>High : 150 - 199<br>Hypertriclyceridemic: 200 - 499<br>Very High : >499 (Enzymatic Colorimetric : GPO-PAP) |
| Note: Above Biological interval is based on 9 to 12 hours fasting | | | | |
| Serum | Cholesterol, HDL | 43.7 | mg/dL | Adult (NCEP ATP-III)<br>Low : < 40<br>High : >=60 (Enzymatic-Colorimetric) |
| Serum | Cholesterol, LDL | 107.0 | mg/dL | Optimal : <100<br>Near or above optimal : 100 - 129<br>Borderline high : 130 - 159<br>High : 160 - 189<br>Very high : >190.<br>(Calculation) |
| Serum | Cholesterol, VLDL | 12.7 | mg/dL | Less than 30<br>(NCEP ATP-III) (Calculation) |
| Serum | Cholesterol/HDL Ratio | 3.7 | | Castelli's Risk Index -I<br>Ideal : <3.5<br>Good: 3.5-5.0<br>High: >=5 (Calculation) |
| Serum | HDL/LDL Ratio | 0.4 | Ratio | Optimal : >0.4<br>Moderate: 0.3-0.4<br>High : <0.3 (Calculation) |
| Serum | LDL/HDL Ratio | 2.4 | Ratio | Castelli's Risk Index -II<br>Ideal : <2.0<br>Good: 2.0-5.0 |

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|---|---|---|---|---|
| | | | | High: >=5 (Calculation) |
| Serum | Non - HDL Cholesterol | 119.7 | mg/dL | Adult (NCEP ATP-III)<br>Optimal : <130<br>Near or above optimal : 130-159<br>Borderline high : 160-189<br>High : 190-219<br>Very high : >220 (Calculation) |
| **Kidney Function** | | | | |
| Serum | Blood Urea Nitrogen (BUN) | 9.8 | mg/dL | 6.0-20.0 (Kinetic Urease/GLDH) |
| Serum | Urea | 20.90 | mg/dL | 16.6 - 48.5 (Kinetic Urease/GLDH) |
| Serum | Creatinine. | 0.46 | mg/dL | 0.60 - 1.10 (Jaffe) |
| Serum | Bun Creatinine Ratio | 21.20 | Ratio | (Calculated) |
| Serum | Est. Glomerular Filtration Rate | 157.0 | | >=18 years : >=60 ml/min/1.73m^2 (MDRD calculation) |

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|---|---|---|---|---|

Estimated Glomerular Filtration Rate (eGFR) test is useful

- Diagnosing and monitoring treatment of acute and chronic kidney diseases (CKD)
- Adjusting dosage of renally excreted medications
- Monitoring kidney transplant recipients
- Estimating glomerular filtration rate for people with chronic kidney disease (CKD) and those with risk factors for CKD (diabetes, hypertension, cardiovascular disease, and family history of kidney disease)

eGFR calculated using the Modification of Diet in Renal Disease (MDRD) Study equation provides a more clinically useful measure of kidney function than serum creatinine alone. This equation takes into account several factors that impact creatinine production, including age, gender, race and Cr method traceable to IDMS. In general, quantification of eGFR values below 60 mL/min/1.73 $m^2$ has more clinical implications for classification of kidney function than values above this level (NIDDK).

**Limitations:**

Creatinine-based estimating equations are not recommended for use with:

- The equation has been validated only for above the age of 18 years with impaired kidney function.
- Individuals with unstable creatinine concentrations like pregnant women; patients with serious co-morbid conditions; and hospitalized patients, particularly those with acute renal failure. Creatinine-based estimating equations should be used only for patients with stable creatinine concentrations.
- Persons with extremes in muscle mass and diet including but is not limited to, individuals who are amputees, paraplegics, or bodybuilders, or have obesity; patients who have a muscle-wasting disease or a neuromuscular disorder; and those suffering from malnutrition, eating a vegetarian or low-meat diet, or taking creatine dietary supplements.

**Interpretation:**

According to the Kidney Disease: Improving Global Outcomes (KDIGO) CKD work group, chronic kidney disease (CKD) is defined as the abnormalities of kidney structure or function, present for more than 3 months, with implications for health. KDIGO guidelines provide the following GFR categories:

| Stage | Terms | eGFR mL/min/1.73 m^2 |
|---|---|---|
| **G1*** | Normal or high | 90 |
| **G2*** | Mildly decreased | 60 to 89 |
| **G3a** | Mildly to moderately decreased | 45 to 59 |
| **G3b** | Moderately to severely decreased | 30-44 |
| **G4** | Severely decreased | 15-29 |
| **G5** | Kidney failure | <15 |

*In the absence of evidence of kidney damage, neither G1 nor G2 fulfill criteria for CKD.

| | | | | |
|---|---|---|---|---|
| Serum | Uric Acid. | 3.40 | mg/dL | 2.60 - 6.00 (Uricase/peroxidase) |
| Serum | Calcium | 8.8 | mg/dL | Adults : 8.6 - 10.3 (End point : Arsenazo III) |

**Liver Function, Profile**

| | | | | |
|---|---|---|---|---|
| Serum | Bilirubin, Total | 1.02 | mg/dL | 0.30 - 1.20 (Colorimetric : Diazo) |
| Serum | Bilirubin, Direct | 0.35 | mg/dL | 0 - 0.2 (Colorimetric : Diazo) |

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| Serum | Bilirubin, Indirect | 0.67 | mg/dL | 0.1 - 1.0 (Calculated) |
| Serum | Aspartate aminotransferase (AST/SGOT) | 16.40 | U/L | Less than 32 (IFCC without P5P) |
| Serum | Alanine aminotransferase (ALT/SGPT) | 12.50 | U/L | 0.00 - 33.00 (IFCC without P5P) |
| Serum | Alkaline phosphatase | 87.3 | U/L | 35.0 - 104.0 (Colorimetric : IFCC) |
| Serum | Gamma Glutamyl-Transferase (GGT) | 17.6 | U/L | 6.0 - 42.0 (Colorimetric : IFCC) |
| Serum | Total Protein. | 7.21 | g/dL | 6.40 - 8.30 (Colorimetric-Biuret) |
| Serum | Albumin. | 4.47 | g/dL | 3.50 - 5.20 (Colorimetric: Bromocresol Green) |
| Serum | Globulin. | 2.74 | g/dL | 2.0-3.9 (Calculated) |
| Serum | Albumin/Globulin | 1.6 | Ratio | (Calculated) |
| Serum | SGOT/SGPT | 1.3 | Ratio | Upto 1.7 (Calculation) |
| **Transferrin Saturation** | | | | |
| Serum | IRON | 79.4 | ug/dl | 50.0 - 170.0 (Colorimetric) |
| Serum | UIBC | 238.3 | ug/dl | 135.0 - 392.0 (FerroZine) |
| Serum | TIBC | 317.70 | ug/dl | 250.00 - 425.00 (Calculation) |
| Serum | TRANSFERRIN SATURATION | 25.0 | % | 15.0 - 50.0 (Calculation) |

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| **IMMUNOLOGY** | | | | |
| **Health Package - 4** | | | | |
| **Thyroid function tests** | | | | |
| Serum | T3 | 1.32 | ng/ml | Adult : 0.800 - 2.00<br><br>Healthy Pregnancy<br>Trimester I : 0.81-1.9<br>Trimester II & III : 1-2.6 (ECLIA) |
| Serum | T4 | 9.36 | ug/dl | 5.10 - 14.10 (ECLIA) |
| Serum | TSH | 1.520 | uIU/ml | Adult : 0.270 - 5.350<br>ATA Guidelines for Healthy Pregnant Women<br>First Trimester : 0.1 - 2.5<br>Second Trimester: 0.2 - 3.0<br>Third Trimester : 0.2 - 3.0 (ECLIA) |

**Note:** TSH has a diurnal rhythm, peaks at 2.00-4.00 am and has lowest level at 5.00-6.00 pm with ultradian variation. Hence thyroid test is only a snapshot of what is occurring within a dynamic system and for treatment purpose, the results should be accessed in conjugation with patient medical history, clinical examination & other tests/finding for confirmation. Many multivitamins (such as Vit B7), supplements (especially hair, skin, and nail) and over-the-counter and prescription medications may affect thyroid test results, and their use should be discussed with the healthcare practitioner prior to testing.
When a high serum TSH concentration and normal free T4 is found, repeat measurement 3-6 months later along with thyroid antibodies after excluding nonthyroidal illness and drug interference is recommended.

| | | | | |
|---|---|---|---|---|
| **Vitamins** | | | | |
| Serum | 25 Hydroxyvitamin D | 13.0 | ng/ml | Deficiency : <= 20<br>Insufficiency: 21 - 29<br>Sufficiency : >= 30 (ECLIA) |

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| **Comments :** Vitamin D is a fat-soluble steroid hormone precursor that is mainly produced in the skin by exposure to sunlight. Vitamin D is biologically inert and must undergo two successive hydroxylations in the liver and kidney to become the biologically active 1,25 - dihydroxyvitamin D. It is commonly agreed that 25-hydroxyvitamin D is the metabolite to determine the overall vitamin D status as it is the major storage form of vitamin D in the human body. This primary circulating form of vitamin D is present human body with levels approximately 1000 fold greater than the circulating 1,25-dihydroxyvitamin D. The half-life of circulating 25-hydroxyvitamin D is 2-3 weeks.<br>**Vitamin D is essential for :** Bone health. In children, severe deficiency leads to bone-malformation, known as rickets. Milder degrees of insufficiency are believed to cause reduced efficiency in the utilization of dietary calcium.<br>**Vitamin D deficiency causes :** Muscle weakness in elderly, the risk of falling has been attributed to the effect of vitamin D on muscle function. Vitamin D deficiency is a common cause of secondary hyperparathyroidism. Elevations of PTH levels, especially in elderly vitamin D deficient adults can result in osteomalacia, increased bone turnover, reduced bone mass and risk of bone fractures. Low vitamin D (25-OH) concentrations are also associated with lower bone mineral density.<br>The results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings. | | | | |
| Serum | VITAMIN B 12 | 474 | pg/ml | 197 - 771 (ECLIA) |
| **Note:** Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath.<br>Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. The test results should always be assessed in conjunction with the patients medical history, clinical examination and other findings. | | | | |
| Serum | Testosterone, Total | 0.210 | ng/ml | 0.084 - 0.481 (ECLIA) |

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**---------- End of the Report ----------**